

SYNC HEALTH

PEPTIDE PATIENT INTAKE

Consolidated Build Specification v1.0

SYNC Health | July 2026 | Confidential | Single Source of Truth

DRAFT

Working draft. Following the July PCAC review, the launch formulary (BPC-157, TB-500, KPV, MOTS-c, Semax, Epitalon) is cleared and live — this draft is clear of PCAC dependency. Governance: compounded peptides are drugs under the FDCA (503A). The intake routes and flags; the provider prescribes.

Clinical review: Dr. Marcus Reid | Product lead: Amelia | Development lead: Patrick

DOCUMENT HIERARCHY

Sources integrated into this version: - Peptide Intake 1.0 clinical build spec (four-layer engine, Dr. Reid sign-off) - SmartStack Build Spec v5.0 — format, question architecture, safety-gate-first sequencing - SYNC Formulation Worksheet — candidate peptide sets per stack - SYNC Formulation Worksheet — candidate peptide sets per stack, post-PCAC (cleared)

Resolution rules: - This is a **drug intake under the FDCA (503A compounding)**, not a supplement quiz. The engine SCORES, FLAGS, and ROUTES. It does NOT prescribe. No product name or dose is shown to the patient pre-consult. - Candidate peptide sets are held in a config layer the engine reads. Following the July PCAC review, BPC-157, TB-500, KPV, MOTS-c, Semax, and Epitalon are cleared and treated as live. Engine logic is catalogue-agnostic. - Where a question could imply a product recommendation, it is reframed as a prescriber-facing signal.

WHAT YOU ARE BUILDING

A personalised patient intake for SYNC's compounded-peptide telehealth line. The user answers ~20–26 questions across 5 sections plus a nervous-system-equivalent readiness module. The intake outputs a **prescriber-facing candidate protocol** (primary stack, secondary signal, active modifiers, gates fired, candidate peptides filtered by exclusions, full chart) delivered into the Bask EMR. A licensed prescriber reviews the chart and makes the clinical decision. The patient sees only their stack DIRECTION pre-consult — never a product or dose.

Internal stack labels (engine-only, NEVER shown to patient): PERFORM · SCULPT · RECOVER · YOUTH · MIND · OPTIMIZE Patient-facing blend names (cleared set): APEX · IGNITE · RESTORE · AEON · [MIND TBD] · DELTA

BUILD ORDER

Complete in this exact sequence. Do not advance until current phase is tested and passing.

Phase 1 — Eligibility & Safety Gates (build first — nothing goes live without these) Phase 2 — Goal & Symptom Routing Phase 3 — Clinical Core + Modifiers Phase 4 — Candidate Protocol Composition (prescriber-facing output) Phase 5 — Bask API Integration & Data Schema

QUIZ FLOW OVERVIEW

PRE-QUIZ: Plain-language medical + telehealth consent ↓ WELCOME SCREEN ↓ SECTION 1: Eligibility & You (6 questions — HARD GATES fire here) Q1 Name → Q2 DOB → Q3 State → Q4 Sex at birth → Q5 Gender → Q6 Pregnancy (natal female only) ↓ [Eligibility confirmed — proceed] SECTION 2: Your Goal (3 questions) Q7 Primary goal → Q8 Secondary goal → Q9 Symptom pattern ↓ [Educational Interstitial #1] SECTION 3: Clinical Core (5 questions — prescriber-mandatory + SOFT GATES) Q10 Height/Weight → Q11 Pre-existing conditions [STANDALONE] → Q12 Conditions → Q13 Medications → Q14 Allergies ↓ [Educational Interstitial #2] SECTION 3B: Going Deeper (goal-branched modifiers, 2–3 per branch) Recovery / Performance / Metabolic / Skin & Longevity / Hormonal branches — each reverse-engineered from peptide indications (2–3 questions) ↓ READINESS MODULE (2 questions — ALL users) ↓ SECTION 4: Labs & History (2–3 questions) Q-Labs bloodwork upload → Q-Inject injectable experience → Q-Surgery recent surgery ↓ [Building animation] RESULTS: Patient sees stack DIRECTION + "prescriber will review." Chart POSTs to Bask.

Total questions: ~20–26 depending on goal branch Estimated completion: 5–6 minutes Progress indicator: Section-based ("Section 3 of 5")

PRE-QUIZ: MEDICAL & TELEHEALTH CONSENT

CRITICAL — must be visible before intake entry, not buried in terms.

"This assessment collects information about your health, goals, medications, and medical history so a licensed healthcare provider can determine whether a compounded peptide protocol is appropriate for you. Completing this assessment is not a guarantee of treatment — a provider reviews every intake and may decline or request more information. Your responses form part of your medical record and are handled under HIPAA. We do not sell your data. By continuing, you consent to a telehealth consultation and to the use of your information for clinical review."

Note: Requires healthcare counsel review before finalisation.

CLINICAL NOTE — Dr. Reid (consent): The "not a guarantee of treatment" line is not optional and it is not soft copy — it is the sentence that makes this an intake for a clinical decision rather than a storefront. Keep it above the fold. A patient must never arrive at the results screen believing they have bought something.

WELCOME SCREEN (NOT A QUESTION)

Note: All copy is working draft. Structure, logic, and data tags are locked. Copy subject to clinical + legal review before build.

"Welcome to your SYNC assessment. This takes about 5 minutes. Your answers go to a licensed provider who reviews whether a personalised peptide protocol is right for you. No guesswork. No off-the-shelf protocols. Let's build something precise — and safe."

AGE ELIGIBILITY DISCLAIMER — must be visible on the welcome screen, above the "Begin" action: "SYNC's peptide programs are available to patients aged 21 and over. By continuing, you confirm you are 21 or older."

- Placement: persistent on welcome screen, not buried in terms. Stated before the patient invests time, and separate from the Q2 hard gate (which enforces it on date of birth).
- Build note: this is a *disclaimer* (visible eligibility statement); Q2 is the *gate* (logic enforcement). Both are required — the disclaimer sets expectation, the gate blocks entry.

CLINICAL NOTE — Dr. Reid (age): 21 is a clinical floor, not a marketing choice — these are growth-signalling and hormonally active compounds and they don't go near a still-maturing endocrine system for a performance or cosmetic goal. State it plainly at the door so no one under 21 invests five minutes only to be turned away, and so the floor reads as the deliberate safety decision it is.

SECTION 1: ELIGIBILITY & YOU

Purpose: Capture identity, fire all HARD eligibility gates before any clinical or commercial step Questions: 6 Time: 1-2 minutes

Q1 — First Name

- Copy: "Let's start with your name."
- Subtext: "We use this to personalise your results — and nothing else."
- Type: Free text
- Data tag: user_name (string)

Q2 — Date of Birth ▲ HARD GATE

- Copy: "What's your date of birth?"
- Subtext: "We need this to confirm eligibility and for your medical record."
- Type: Date
- Data tag: date_of_birth (date) → derive age
- Gate logic: age <21 → HARD BLOCK. Fail copy: "SYNC's peptide programs are available to patients 21 and over."

CLINICAL NOTE — Dr. Reid (Q2): 21, not 18. These are growth-signalling and hormonally active compounds and I won't put them near a still-maturing endocrine system for a cosmetic or performance goal. This is a firm clinical floor, not a risk-posture preference.

Q3 — State of Residence ▲ HARD GATE

- Copy: "Which state do you live in?"
- Subtext: "Our providers are licensed state by state — this confirms we can care for you."
- Type: Single select (dropdown — COVERED STATES ONLY)
- Data tag: state (enum)
- Gate logic: ineligible/uncovered state → HARD BLOCK + waitlist email capture. Fail copy: "We're not yet live with providers in your state. Leave your email and we'll tell you the moment we are."
- Build note: covered-state list is read from Bask provider-network coverage (see \$Integration).

Q4 — Sex Assigned at Birth

- Copy: "What sex were you assigned at birth?"
- Subtext: "This helps your provider dose accurately and read your labs correctly."
- Type: Single select
- Options: Male / Female
- Data tag: sex_at_birth (enum)

Q5 — Gender

- Copy: "How do you describe your gender?"
- Type: Single select
- Options: Woman / Man
- Data tag: gender_identity (enum)

Q6 — Pregnancy Status ▲ HARD GATE

- Copy: "Which best describes you right now?"
- Type: Single select
- Visibility: only if Q4 sex_at_birth = Female
- Data tag: pregnancy_status (enum)

Answer	Data Value	Engine Signal
Not pregnant, breastfeeding, or trying	not_applicable	Proceed
Pregnant	pregnant	HARD BLOCK
Breastfeeding	breastfeeding	HARD BLOCK
Trying to conceive	ttc	HARD BLOCK

Fail copy (any block): "For your safety, we can't offer peptide therapy while pregnant, breastfeeding, or trying to conceive. We'd love to help when the time is right."

SECTION 2: YOUR GOAL

Purpose: Route the patient toward a stack direction and capture presenting signals Questions: 3 Time: 30-60 seconds

Q7 — Primary Goal

- Copy: "[Name], what are you here to work on first?"
- Subtext: "Pick the one that matters most — your provider builds around this."
- Type: Single select
- Data tag: primary_goal (enum)

Answer	Data Value	Engine Signal
Build strength & performance	perform	→ PERFORM primary
Lose fat / reshape my body	sculpt	→ SCULPT primary
Recover from injury / reduce inflammation	recover	→ RECOVER primary
Age well / longevity	youth	→ YOUTH primary
Sharpen focus & mood	mind	→ MIND primary
Sleep & stress	optimize	→ OPTIMIZE primary

Q8 — What's Driving It ■ ROOT-CAUSE (not a secondary goal picker)

- Copy: "When did you first notice this — and what do you think is behind it?"
- Subtext: "You don't need to be sure. Your answer helps us find the root cause, not just treat the surface."
- Type: Single select
- Data tag: root_cause_signal (enum)
- Design intent: this is NOT "what else do you want to work on." These patients present with a problem and often don't know the driver. The intake's job is to surface the underlying cause so SYNC and the provider can direct them — not to hand them a menu. The answer feeds the prescriber's read of root cause and sets a secondary stack signal where clinically implied.

Answer	Data Value	Engine Signal
It came on gradually — I feel like my body is running less efficiently than it used to	systemic_decline	YOUTH + PERFORM secondary signal; age/recovery-capacity read
It started after an injury, surgery, or physical setback	post_injury	RECOVER secondary signal
It tracks with stress — it's worse when I'm under pressure	stress_driven	OPTIMIZE secondary signal; readiness flag
It's tied to my sleep — everything's worse when I'm not sleeping	sleep_driven	OPTIMIZE secondary signal
It followed a metabolic change — weight gain, a plateau, or a medication	metabolic_driven	SCULPT secondary signal; probe GLP-1 history
It came with age or a hormonal shift	hormonal_driven	YOUTH secondary signal
I genuinely don't know — that's why I'm here	cause_unknown	No secondary routing; full prescriber review — this patient needs direction

CLINICAL NOTE — Dr. Reid (Q8): This is the right instinct and I want it protected in the build. A patient selecting "I don't know — that's why I'm here" is not a gap in the data, it's the most honest and most common answer, and it must route to a fuller provider review rather than being penalised for indecision. We are the authority here. The intake's job is to hand me a root-cause hypothesis, not to make the patient self-diagnose. Do not turn this back into a goal menu.

Q9 — Symptom Pattern

- Copy: "Which of these sound most like you right now?"
- Subtext: "Pick the ones that feel true, even if you can't fully explain them."
- Type: Multi-select (max 3)
- Data tag: symptom_pattern (array)

Answer	Data Value	Engine Signal
Slow recovery — training or injuries linger	slow_recovery	RECOVER + PERFORM signal
Stubborn weight or a plateau I can't break	weight_plateau	SCULPT signal
Low energy / crashing through the day	low_energy	PERFORM + OPTIMIZE signal
Poor sleep / waking unrested	poor_sleep	OPTIMIZE signal
Brain fog / low motivation	brain_fog	MIND signal
Joint pain or an old injury that won't settle	joint_pain	RECOVER signal
Feeling older than my age	feeling_aged	YOUTH signal (never routes alone)

CLINICAL NOTE — Dr. Reid (Q9): "Feeling older than my age" is a real presentation but it's diffuse — it must never route a patient into a stack on its own. It's a supporting signal to the goal they actually chose in Q7, not a driver. Build it that way.

EDUCATIONAL INTERSTITIAL #1

"Peptides aren't a shortcut — they're a signal. They tell your body to repair, recover, or regulate more efficiently. Which signal you need depends on what's actually going on underneath. That's what the next few questions figure out — and why a provider, not an algorithm, makes the final call."

SECTION 3: CLINICAL CORE

Purpose: Build the medical record. Fire SOFT gates. Prescriber-mandatory. Questions: 5 Time: 2 minutes

Q10 — Height & Weight

- Copy: "What's your height and current weight?"
- Subtext: "Your provider uses this for dosing and to track your progress."
- Type: Number (height ft/in + cm toggle; weight lb/kg toggle)
- Data tag: height (number), weight (number) → derive BMI (not shown to patient)

Q11 — Pre-Existing Medical Conditions (Serious/History) ▲ GATE — STANDALONE

- Copy: "Do you have — or have you ever had — any serious medical conditions we should know about?"
- Subtext: "This is important for your safety. Some peptides aren't appropriate depending on your medical history."
- Type: Single select (per condition) — presented as a short focused screen, kept separate from the day-to-day conditions in Q12
- Data tag: preexisting_history (enum) [retains malignancy_history mapping in engine]
- Design intent: reframed from a cancer-only question to a broader serious/history screen, but it STILL captures malignancy status explicitly — that screen is the load-bearing safety gate and is never merged into Q12's checklist.

Answer	Data Value	Engine Signal
No serious or historical conditions	preexisting_none	Proceed
Cancer — currently diagnosed or in treatment	cancer_active	▲ HARD BLOCK pending clinical call. No auto-approve.
Cancer — in remission	cancer_remission	▲ SOFT GATE — mandatory prescriber review; EXCLUDE GH-axis candidates from auto-populated set
Cancer — in the past	cancer_past	▲ SOFT GATE — mandatory prescriber review; EXCLUDE GH-axis candidates from auto-populated set
Other serious condition (current or historical) — I'll describe it	serious_other	▲ SOFT GATE — mandatory prescriber review + free text

- Free-text field renders if serious_other selected · Data tag: preexisting_other_detail (string)

CLINICAL NOTE — Dr. Reid (Q11 — the one I will not soften): Broadening the wording to "serious medical conditions" is fine — better, even, because it catches history I'd want anyway — but the malignancy options stay explicit inside it and this screen stays standalone. Active cancer is a **HARD** block: the GH-secretagogue peptides raise IGF-1, IGF-1 is mitogenic, and I am not putting a growth signal into someone with active disease, full stop. Remission and history are **SOFT** so we don't wrongly turn away a large, well population, but the engine must strip every GH-axis candidate from what I see pre-loaded. Do not fold this into Q12's checklist — a malignancy question buried in a list of tick-boxes is exactly how these get missed.

Q12 — Diagnosed Conditions ▲ SOFT GATES

- Copy: "Have you been diagnosed with any of the following?"
- Type: Multi-select
- Data tag: diagnosed_conditions (array)

Answer	Data Value	Gate / Action
Cardiovascular disease / heart condition	cvd	SOFT — review; exclude GH-axis pending review
Diabetes (Type 1 or 2)	diabetes	SOFT — review; gates GLP-1 AND GH-secretagogue candidates (GH is diabetogenic — provider needs HbA1c)
Kidney disease	kidney	SOFT — review; affects dosing & candidate set
Liver disease	liver	SOFT — review; affects dosing & candidate set
Thyroid disorder	thyroid	Modifier flag (see M-notes) + review
Autoimmune condition	autoimmune	NOTE for provider
Bleeding or clotting disorder	bleeding	SOFT — review
Pancreatitis (current or past)	pancreatitis	SOFT — EXCLUDE GLP-1 candidates
Diabetic eye disease / retinopathy	retinopathy	SOFT — EXCLUDE rapid-titration GH-axis pending review
Mental health condition	mental_health	NOTE for provider; no candidate change
High blood pressure	hypertension	NOTE for provider
None of the above	none	Proceed

CLINICAL NOTE — Dr. Reid (Q12): Two of these — pancreatitis and retinopathy — weren't in the first draft and their absence would have been a real gap. Pancreatitis excludes GLP-1 candidates; that's standard of care. Retinopathy excludes rapid IGF-1 elevation because it can worsen proliferative disease. Diabetes gates BOTH the GLP-1 and the GH-secretagogue logic — GH is diabetogenic and I need the HbA1c before anything in that class goes near them. Thyroid I want surfaced as a modifier, not just a condition — subclinical thyroid dysfunction is the single most common thing masquerading as a metabolic or energy complaint in this cohort.

Q13 — Current Medications

- Copy: "List any prescription or over-the-counter medications you take."
- Subtext: "Include anything regular — this is how your provider checks for interactions."
- Type: Multi-select quick-pick + free text
- Data tag: medications (array + string)
- Quick-pick: GLP-1s · blood thinners · thyroid medication · antidepressants/SSRIs · blood pressure medication · insulin · corticosteroids · hormone therapy/TRT · "None"
- Engine: thyroid med → thyroid modifier; hormone therapy/TRT → hormone_therapy_active; GLP-1 → glp1_active cross-modifier; any entry → prescriber interaction review

Q14 — Allergies

- Copy: "Do you have any allergies we should know about?"
- Subtext: "Including reactions to injections, preservatives, or medications."
- Type: Multi-select + free text

- Data tag: allergies (array + string)
- Options surface injectable-relevant: benzyl alcohol · metacresol · latex · "No known allergies" + free text
- Engine: benzyl alcohol / metacresol → SOFT gate, exclude affected injectable formulations

EDUCATIONAL INTERSTITIAL #2

"Everything you've just told us goes to a licensed provider — not a bot. They read your history before anything is offered. That's the difference between a peptide protocol and a supplement you buy off a shelf: someone qualified is accountable for it."

SECTION 3B: GOING DEEPER (MODIFIER BRANCHES)

Purpose: Capture prescriber-relevant modifiers specific to the chosen goal Branch logic: only the branch matching Q7 primary_goal appears Questions: 2-3 per branch [RX] = prescriber-relevant clinical data · other = enrichment

METHODOLOGY — read before editing any branch. These questions are reverse-engineered from the actual peptide indications in the SYNC formulary. Each answer describes a *patient presentation in their own words* and carries a hidden candidate_signal that maps to the compound whose indication matches that presentation. The patient never sees a peptide name or a benefit claim — that would breach 503A promotion rules. The mapping lives in the engine; the patient only sees themselves described. When the formulary changes, update the candidate_signal targets, not the patient-facing copy.

RECOVERY BRANCH — fires if primary_goal = recover

Formulary basis: BPC-157 (systemic soft-tissue + gut repair) · TB-500 (cell migration, soft-tissue) · KPV (mucosal anti-inflammatory) · GHK-Cu (collagen/connective-tissue remodelling) · RESTORE blend (all four). All cleared and live.

Q-Rec-1: What's the recovery picture [RX] - Copy: "Which of these is closest to what's going on?" - Subtext: "There's no wrong answer — this helps your provider target the right approach." - Type: Single select - Data tag: recovery_presentation (enum)

Answer	Data Value	Candidate Signal (engine-only)
A specific injury or surgery I'm trying to heal	acute_injury	Soft-tissue repair candidates (BPC-157/TB-500 class)
I train hard and my body isn't recovering fast enough between sessions	training_recovery_deficit	Soft-tissue + GH-axis recovery candidates
Gut inflammation or a gut issue that won't settle	gut_inflammation	Gut-repair / mucosal candidates (BPC-157/KPV class)
Ongoing pain or inflammation across several areas	systemic_inflammation	Comprehensive repair blend (RESTORE class)
Ageing joints, skin, or connective tissue	connective_tissue_aging	Connective-tissue/collagen candidates (GHK-Cu class)

Q-Rec-2: How long [RX] - Copy: "How long has this been going on?" - Type: Single select - Data tag: recovery_duration (enum) - Options: Recent — an acute thing / A few months / A year or more / It comes and goes - Engine: chronic → higher-intensity candidate flag for prescriber

Q-Rec-3: Training load [RX] - Copy: "How hard are you training right now?" - Type: Single select - Options: Not training / Light / Moderate / High / Elite or competitive - Data tag: training_load (enum) → modifier: high_training_load if high/elite (maps the 6x/week presentation)

PERFORMANCE BRANCH — fires if primary_goal = perform

Formulary basis: Sermorelin (single, GH pulse) · CJC-1295 + Ipamorelin = APEX (dual GH-axis pulse → recovery, body composition, sleep). GH-secretagogue class — subject to Q11/Q12 GH-axis exclusions.

Q-Perf-1: What you're chasing [RX] - Copy: "What's the main thing you want to shift?" - Type: Single select - Data tag: performance_presentation (enum)

Answer	Data Value	Candidate Signal (engine-only)
Recovery between training sessions	recovery_focus	GH-axis recovery candidates (Sermorelin / APEX)
Body composition — leaner, more muscle definition	body_comp_focus	GH-axis body-composition candidates
Sleep quality and how I feel the next day	sleep_quality_focus	GH-axis / sleep-linked candidates
I used to perform at a higher level and want it back	performance_decline	Full GH-axis review

Q-Perf-2: Training load [RX] - Copy: "How hard are you training right now?" - Type: Single select - Options: Light / Moderate / High / Elite or competitive - Data tag: training_load (enum) → modifier: high_training_load if high/elite

Q-Perf-3: Sleep [RX] - Copy: "How's your sleep and recovery at the moment?" - Type: Single select - Options: Good / Broken / Never feel rested / Not something I track - Data tag: sleep_state (enum) — GH-axis peptides affect sleep architecture; prescriber-relevant

METABOLIC BRANCH — fires if primary_goal = sculpt

Formulary basis: AOD-9604 (targeted lipolysis, no GH-axis/glucose effect) · Tesamorelin (visceral-fat specific, GH/IGF-1 mediated) · MOTS-c (mitochondrial/insulin sensitivity — cleared, live) · GLP-1 as standalone 503A, never blended. SCULPT/IGNITE blend avoids GLP-1.

Q-Metab-1: The metabolic picture [RX] - Copy: "Which of these sounds most like your situation?" - Subtext: "Be honest — this helps your provider pick the right path, not judge." - Type: Single select - Data tag: metabolic_presentation (enum)

Answer	Data Value	Candidate Signal (engine-only)
I'm doing the work but the fat won't shift	resistant_fat	Lipolysis candidates (AOD-9604 class)
It's specifically stubborn belly / abdominal fat	visceral_fat	Visceral-fat candidates (Tesamorelin class)
My metabolism feels slow — I've plateaued	metabolic_slowdown	Mitochondrial/metabolic candidates (MOTS-c class)
I want appetite and food-noise support	appetite_focus	GLP-1 standalone review (503A prescriber decision)
I've regained weight after stopping a GLP-1	glp1_rebound	Peptide-native maintenance review

Q-Metab-2: GLP-1 history [RX] - Copy: "Have you used a GLP-1 medication (Ozempic, Wegovy, Mounjaro, compounded semaglutide)?" - Type: Single select - Options: Currently → glp1_active · Previously → glp1_experienced · Never → glp1_none - Data tag: glp1_use (enum) → cross-modifier; pancreatitis gate (Q12) excludes GLP-1 candidates

Q-Metab-3: Goal weight [RX] - Copy: "What's your goal weight?" - Type: Number → modifier: high_deficit_target if >15% delta from Q10 weight

SKIN & LONGEVITY BRANCH — fires if primary_goal = youth

Formulary basis: GHK-Cu (skin remodelling, collagen — live) · Epitalon (circadian/longevity — cleared, live) · NAD+ (cellular energy, redox — live) · AEON blend (Epitalon/GHK-Cu).

Q-Long-1: What's front of mind [RX] - Copy: "When you think about ageing well, what matters most right now?" - Type: Single select - Data tag: longevity_presentation (enum)

Answer	Data Value	Candidate Signal (engine-only)
Skin — firmness, texture, visible ageing	skin_focus	Skin-remodelling candidates (GHK-Cu class)
Energy — I don't have the cellular drive I used to	cellular_energy	Cellular-energy candidates (NAD+ class)
Sleep and my body clock feel off	circadian_focus	Circadian candidates (Epitalon class)
General healthspan — I want to age slower overall	broad_longevity	Full longevity review (AEON class)

Q-Long-2: Recent bloodwork (enrichment) - Copy: "When did you last have bloodwork done?" - Type: Date · optional

HORMONAL HEALTH BRANCH — fires if primary_goal = optimize (women's hormonal lane)

Formulary basis: PT-141 / Bremelanotide (melanocortin-pathway libido/arousal). Distinct from the sleep/stress OPTIMIZE lane — see note.

Q-Horm-1: What you're noticing [RX] - Copy: "What would you most like to feel a change in?" - Subtext: "This is private and goes only to your provider." - Type: Single select - Data tag: hormonal_presentation (enum)

Answer	Data Value	Candidate Signal (engine-only)
Low libido or desire	low_desire	Libido candidates (PT-141 class)
Arousal or responsiveness	arousal_focus	Arousal candidates (PT-141 class)
Both	libido_arousal_both	Full hormonal review

BRANCH NOTE — OPTIMIZE is doing double duty. The internal OPTIMIZE label currently spans two different patient intents: (a) sleep/stress and (b) women's hormonal/libido (PT-141). These are clinically unrelated and should be split into two goals at Q7 before launch — "Sleep & stress" and "Intimacy & hormonal health" — so the right branch fires. Flagged for Amelia + Dr. Reid. Until split, Q7 "Sleep & stress" routes here only if sex_at_birth logic and a disambiguating sub-question confirm intent.

CLINICAL NOTE — Dr. Reid (all branches): This is the right way round — we describe the patient's problem and let the engine find the molecule, rather than advertising the molecule and asking the patient to self-select it. Two guardrails I want held. First, no branch answer may ever display the candidate it maps to — the moment a patient sees "this points to BPC-157" we've made a promotional claim on a compounded drug. Second, every candidate signal still passes through the Q11/Q12 gates before it reaches me: a training-recovery answer that maps to a GH-axis candidate must still be stripped if that patient flagged malignancy history. The branch tells me what they're presenting with; the gates tell me what I'm not allowed to give them. Both run, every time.

READINESS MODULE (ALL USERS, REGARDLESS OF GOAL)

Placement: after Section 3B, before Section 4. Purpose: captures a baseline stress/recovery signal for the prescriber — the peptide-flow equivalent of SmartStack's nervous-system module. Does NOT score into a product.

Q-Ready-1: Default State

- Copy: "When life is reasonably normal — no particular crisis — how does your body usually feel?"
- Type: Single select
- Data tag: baseline_state (enum)

Answer	Data Value	Prescriber Signal
Calm and settled most of the time	regulated	none
Slightly on edge / always braced	low_activation	note
Frequently tense or anxious without clear reason	moderate_dysregulation	note — recovery capacity flag
Chronically stressed — can't remember feeling relaxed	severe_dysregulation	flag — provider reviews readiness before GH-axis protocol

Q-Ready-2: Recovery Speed

- Copy: "After a stressful week, how long until you feel like yourself again?"
- Type: Single select
- Data tag: recovery_rate (enum)
- Options: A few hours (fast) / By next morning (moderate) / Several days (slow) / I never fully reset (no_recovery)
- Engine: slow/no_recovery → recovery-capacity flag on chart

CLINICAL NOTE — Dr. Reid (Readiness Module): This isn't a scoring input and it must not become one — it's context I read before I decide how hard to push a protocol. Someone chronically dysregulated with no recovery capacity is not a candidate for an aggressive GH-axis stack on day one regardless of what their goal says, and I want that visible on the chart. It routes nothing. It informs me.

SECTION 4: LABS & HISTORY

Purpose: final clinical enrichment + injectable readiness Questions: 2-3 Time: 1 minute

Q-Labs — Bloodwork Upload (enrichment, incentivised)

- Copy: "Have recent bloodwork? Upload it for a faster, better-dosed protocol."
- Subtext: "Optional — but it helps your provider personalise faster."
- Type: File upload · Data tag: bloodwork_file (file) · optional

Q-Inject — Injectable Experience [RX]

- Copy: "Have you used injectable medications or peptides before?"
- Type: Single select
- Data tag: injectable_experience (enum)
- Options: Yes, comfortable / Yes, but nervous / No, first time → modifier: injectable_naive
- Engine: injectable_naive cross-modifier — prescriber paces onboarding

CLINICAL NOTE — Dr. Reid (Q-Inject): First-injection experience drives compliance more than people expect. I need injectable_naive visible so I can pace onboarding and set expectations — it changes how I write the first protocol, not whether I write one.

Q-Surgery — Recent Surgery/Hospitalization [RX]

- Copy: "Any surgery or hospital stay in the past 12 months?"
- Type: Yes + free text / No
- Data tag: recent_surgery (bool + string)

RESULTS — PATIENT-FACING VS PRESCRIBER-FACING

Patient-facing screen (pre-consult — the ONLY thing the patient sees)

- Building animation.
- "[Name], here's your focus area: **[patient-facing stack name]**"
- Plain-language why, tied to their own Q7/Q9 answers.
- "A SYNC-affiliated provider will review your intake and confirm your personalised protocol. You'll hear from us shortly."
- **NO product name. NO peptide. NO dose. NO price tied to a compound.**

CLINICAL NOTE — Dr. Reid (patient results): No molecule and no dose on this screen, ever. The moment a patient sees "BPC-157, 250mcg" before I've reviewed them, this stops being telehealth and becomes a vending machine — and it becomes indefensible. Stack DIRECTION only. The provider closes the gap.

Prescriber-facing chart (POSTs to Bask EMR)

```
PATIENT: [name, age, state, sex_at_birth, gender_identity]
PRIMARY STACK: [e.g. SCULPT]
SECONDARY SIGNAL: [e.g. OPTIMIZE]
SYMPTOM PATTERN: [array]
MODIFIERS ACTIVE: [glp1_experienced, high_deficit_target, injectable_naive, thyroid, ...]
READINESS: [baseline_state, recovery_rate — flags if dysregulated/no_recovery]
GATES FIRED: [none / soft: cardiovascular review, pancreatitis-GLP-1 excluded, ...]
CANDIDATE PEPTIDES: [from config — EXCLUDING anything a fired gate removes]
FLAGS FOR REVIEW: [malignancy history / med interaction / retinopathy / ...]
FULL INTAKE RECORD: [height/weight/BMI, conditions, meds, allergies, labs, goals, free-text, surgery]
```

Build requirement: the engine must NEVER surface a candidate peptide that a fired gate excludes. Gate filtering runs BEFORE candidate population.

CANDIDATE PEPTIDE CONFIG (NOT ENGINE LOGIC — ENGINE READS THIS)

Following the July PCAC review, all launch candidates are cleared and live. The intake is clear of PCAC dependency.

Internal stack	Candidate set	Status
RECOVER	BPC-157, TB-500, KPV, GHK-Cu	LIVE
PERFORM	Sermorelin; CJC-1295 + Ipamorelin	LIVE
SCULPT	AOD-9604, Tesamorelin, MOTS-c (peptide-native)	LIVE
YOUTH	NAD+, Glutathione, GHK-Cu, Epitalon	LIVE
MIND	Selank, Semax	LIVE
OPTIMIZE	(sleep/stress candidate — HOLD, see note)	OPEN

CLINICAL NOTE — Dr. Reid (config): With the July review cleared, the launch formulary is live — but two positions hold regardless of clearance. SCULPT stays peptide-native — AOD-9604, Tesamorelin, MOTS-c — not compounded GLP-1s; blending compounded GLP-1s with MOTS-c is an active FDA enforcement target and I won't put SYNC's clinical name behind it. A GLP-1, if needed, is a standalone 503A provider decision, not something the intake surfaces. And OPTIMIZE still has no clean candidate now that DSIP is out — leave the lane's routing live but HOLD its candidate set. An empty candidate lane is honest; forcing a GH secretagogue into a sleep lane is a mismatch patients will feel. Clearance changed the regulatory status, not either of these clinical calls.

COMPLETE DATA SCHEMA

Eligibility & Identity | Tag | Type | Source | |---|---|---| | user_name | string | Q1 | | date_of_birth | date | Q2 | | state | enum | Q3 | | sex_at_birth | enum | Q4 | | gender_identity | enum | Q5 | | pregnancy_status | enum | Q6 (natal female) |

Goal & Symptom | Tag | Type | Source | |---|---|---| | primary_goal | enum | Q7 | | root_cause_signal | enum | Q8 | | symptom_pattern | array | Q9 |

Clinical Core | Tag | Type | Source | |---|---|---| | height / weight | number | Q10 | | malignancy_history | enum | Q11 | | diagnosed_conditions | array | Q12 | | medications | array + string | Q13 | | allergies | array + string | Q14 |

Modifiers (branch-dependent) | Tag | Type | Source | |---|---|---| | recovery_presentation | enum | Q-Rec-1 | | recovery_duration | enum | Q-Rec-2 | | performance_presentation | enum | Q-Perf-1 | | sleep_state | enum | Q-Perf-3 | | metabolic_presentation | enum | Q-Metab-1 | | longevity_presentation | enum | Q-Long-1 | | hormonal_presentation | enum | Q-Horm-1 | | glp1_use | enum | Q-Metab-2 | | goal_weight | number | Q-Metab-3 | | training_load | enum | Q-Rec-3 / Q-Perf-2 | | candidate_signal | array | derived per branch — engine-only, never displayed |

Readiness & History | Tag | Type | Source | |---|---|---| | baseline_state | enum | Q-Ready-1 | | recovery_rate | enum | Q-Ready-2 | | bloodwork_file | file | Q-Labs | | injectable_experience | enum | Q-Inject | | recent_surgery | bool + string | Q-Surgery |

Engine Output (Audit Trail) | Tag | Type | Notes | |---|---|---| | computed_primary_stack | enum | From Q7 + Q9 signals | | computed_secondary_stack | enum | From Q8 + Q9 | | active_modifiers | array | All modifier flags | | gates_fired | array | Hard + soft gates triggered | | candidate_peptides | array | Post gate-exclusion candidate set | | flags_for_review | array | Prescriber review flags | | eligibility_status | enum | pass / hard_block / waitlist |

BASK API INTEGRATION

Three components the developer builds: 1. **Intake front-end** — questions/branching/UI above, SYNC brand & domain. Hard gates (Q2/Q3/Q6) fire client-side to fail before any commercial step. 2. **Scoring & routing engine** — pure function: answers → {computed stacks, modifiers, gates, candidate_peptides post-exclusion, chart}. Gate-exclusion runs before candidate population. 3. **Bask integration** — POST chart to Bask patient/intake endpoint → lands in prescriber queue; READ covered-state list from Bask for Q3 dropdown.

Dependencies to confirm from Bask API docs before building component 3: - [] Patient/intake creation endpoint schema — accepted fields + required chart format - [] Whether state eligibility is queried from Bask or maintained SYNC-side - [] File-upload handling for bloodwork (Q-Labs) — Bask-side or SYNC-side storage

PRE-LAUNCH CHECKLIST

Phase 1 — Eligibility & Safety Gates (CRITICAL — build first) - [] Q2 age <21 HARD block - [] Q3 state gate reads Bask covered-state list; ineligible → block + waitlist - [] Q6 pregnancy HARD block (natal female) - [] Q11 cancer STANDALONE; active → hard block; remission/past → soft gate + GH-axis exclusion - [] Q12 soft gates: diabetes gates GLP-1 + GH-secretagogue; pancreatitis excludes GLP-1; retinopathy excludes rapid GH-axis; excipient allergy excludes affected formulations - [] Gate-exclusion filter runs BEFORE candidate population (engine must never surface an excluded candidate)

Phase 2 — Goal & Symptom Routing - [] Q7 primary goal routing; Q9 "feeling_aged" never routes alone - [] Q8 secondary weighting lower than primary

Phase 3 — Clinical Core + Modifiers - [] Q10-Q14 clinical core writing to chart - [] Thyroid surfaces as modifier + condition - [] Branch modifiers (Recovery/Performance/Metabolic/Skin&Longevity/Hormonal) fire on correct primary_goal; candidate_signal never displayed to patient; every signal passes Q11/Q12 gates before reaching prescriber - [] Readiness module (Q-Ready-1/2) flags dysregulation/no_recovery on chart - [] injectable_naive cross-modifier captured (Q-Inject)

Phase 4 — Candidate Protocol - [] Patient-facing screen shows stack DIRECTION only — no molecule, no dose, no price - [] Prescriber chart complete and POSTs full record - [] Candidate config live (all launch peptides PCAC-cleared); OPTIMIZE candidate on HOLD

Phase 5 — Data Schema & Integration - [] All data tags writing to profile - [] Bask endpoint schema confirmed; chart POST succeeds to prescriber queue - [] Covered-state list read from Bask - [] Bloodwork upload handling confirmed

ITEMS PENDING BEFORE GO-LIVE

- PCAC review — CLEARED (BPC-157, TB-500, KPV, MOTS-c, Semax, Epitalon live). No outstanding regulatory gate on the launch formulary.
- RESTORE/AEON/IGNITE/DELTA/APEX + MIND name → trademark + domain clearance confirmed
- OPTIMIZE sleep/stress candidate → Dr. Reid to confirm candidate set (DSIP out; lane routing live, candidate on HOLD)
- Consent copy → healthcare counsel sign-off
- Bask API contract → three dependencies above confirmed

SYNC Peptide Patient Intake — Consolidated Build Spec v1.0 — SYNC Health — July 2026 Clinical review: Dr. Marcus Reid. Product lead: Amelia. Development lead: Patrick. "Build in this order: eligibility and safety gates first, routing second, clinical core third, candidate protocol last. The engine routes and flags. The provider prescribes. That line does not move." — Dr. Reid

SYNC Health | Peptide Intake Build Spec v1.0 | July 2026 CONFIDENTIAL — FOR PATRICK